

The Concept of 'Homeopathic Vaccines' Is Not Rational and Lacks Evidence: A Commentary on the Paper by Loeb et al, 2018

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In 2018, Loeb et al published a double-blind, randomised trial on 150 students of McMaster University, Hamilton, Canada.¹ They assigned participants into three groups: (1) 'homeopathic vaccines' of diphtheria, pertussis, tetanus, mumps and measles; (2) placebo; or (3) conventional diphtheria, pertussis, tetanus (Tdap) and mumps, measles, rubella (MMR) vaccines. Following administration, there was significant increase in the mean titres of immunoglobulin G (IgG) antibody from baseline for conventional vaccine antigens ($p < 0.001$ for each), but none for the response to homeopathic vaccines or placebo. The authors concluded that, in contrast to conventional vaccines, homeopathic vaccines do not evoke antibody responses but produce a response that is similar to placebo.

There were controversial elements in the adopted study design:

1. The homeopathic therapeutic system is based on the law of 'similars': that is, '*Similia Similibus Curentur*',² which means 'let likes be treated by likes'. Medicine selection in homeopathy is based on signs and symptoms of the diseased individual. Prevention of epidemic disease by homeopathic medicine ('genus epidemicus') may be possible with the help of signs and symptoms produced by a particular epidemic disease at a particular time and place, and in a particular population.³ If the symptoms vary, medicines will also vary. Epidemic diseases may be recurrent, where presenting symptoms vary in different times and/or locations, or they may be non-recurrent, where presenting symptoms are mostly fixed. Hahnemann used *Belladonna* as the 'genus epidemicus' for scarlet fever in the year 1801 and *Aconitum napellus* for a kind of purpura miliaris.⁴ But in

both cases, the 'genus epidemicus' was selected for a particular disease or condition (i.e. symptoms-specific, not specific for nosological diagnosis). This concept is not reflected in the trial design adopted by Loeb et al.

2. In their trial, the researchers selected specific nosodes empirically for preventing a specified disease condition; for example, *Diphtherinum* for diphtheria, without consideration of individualised signs and symptoms. It is not the homeopathic concept per se, because here the signs and symptoms of the epidemic were not taken into account to prescribe a medicine: rather the concept was 'isopathic', not 'homeopathic'.
3. The selection by Loeb et al of potencies in 30cH, 200cH and 1000cH, and their administration on three consecutive days, remains empirical and unexplained. This manner of empirical selection of potencies and repetitions does not conform to the principles of homeopathy.

The concept of 'homeopathic vaccines' is not only irrational: it is also not supported by any adequate research evidence. This critique may be taken into account in the design of future clinical prophylaxis trials in homeopathy.

Authors' Contributions

SD and MK contributed to conception of the commentary, the literature search and preparation of the manuscript. All authors reviewed, edited and approved the final manuscript.

Conflict of Interest

None declared.

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